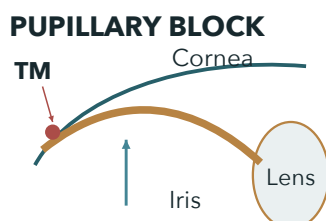


Secondary and developmental outflow

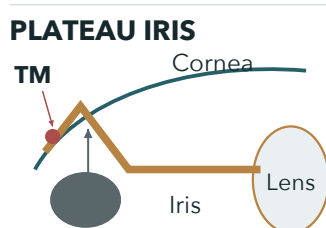
TYPE B | Decide whether tissue is crowded, blocked or malformed.

Peripheral crowding has different causes

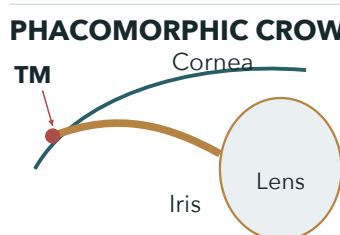
Three angle anatomies



Pressure difference
bows iris forward



Ciliary process supports
peripheral iris; centre flat



Enlarged lens
shallows the chamber

A flat central iris and persistent peripheral crowding after a patent iris opening favour plateau iris. The ciliary processes are the key support.

Same contact, different initiating lesion

Pattern	Primary driver
Pupillary block	Posterior chamber pressure bows peripheral iris forward
Plateau iris	Anterior ciliary processes elevate the peripheral iris
Phacomorphic crowding	Enlarged lens displaces iris and shallows the chamber

Developmental outflow failure

Malformed drainage tissue raises IOP in congenital glaucoma. The infant globe is relatively distensible: enlargement, corneal oedema and Descemet breaks can accompany pressure-related stretching.

Sources Pavlin and Foster: Plateau iris imaging study | NEI: Types of glaucoma | Ueno et al.: Phacolytic trabecular microscopy

An open angle can still drain poorly

Secondary obstruction map

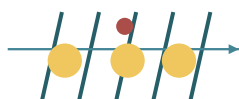
OPEN ANGLE, IMPAIRED FLOW

Steroid response



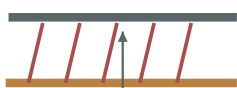
Drainage tissue changes
raise resistance

Phacolytic protein + cells



Lens material and
macrophages clog spaces

NEOVASCULAR MEMBRANE



Retinal ischaemia → VEGF
Angle membrane → contraction

The material and its site matter. Neovascular disease can start by covering drainage tissue and progress to contraction with synechial closure.

Cause → obstruction

Cause	High-yield mechanism
Steroid response	Altered drainage tissues increase conventional resistance
Phacolytic glaucoma	Lens protein plus macrophages obstruct trabecular spaces
Neovascular glaucoma	Ischaemia → VEGF → iris/angle fibrovascular membrane
Congenital glaucoma	Developmentally abnormal outflow tissue

Structure is not a pressure label

Glaucoma injures retinal ganglion-cell axons at the optic nerve head. High IOP increases risk, but normal-range IOP does not exclude glaucomatous damage; assess the rim, nerve fibre layer and corresponding field.

Sources NEI: Steroid-induced outflow resistance | Ueno et al.: Phacolytic trabecular microscopy | NEI: Types of glaucoma